

Policy Document 1: Specialized Advanced Imaging
Clinical Policy: Outpatient Lumbar Spine MRI (CPT 72148)
Effective Date: March 1, 2026

Review Cycle: Annual

I. Policy Statement

Prior authorization is strictly mandatory for all non-emergent, outpatient Magnetic Resonance Imaging (MRI) scans of the lumbar spine. This policy applies to all commercial and managed care plan members. Imaging performed in an emergency department or during an inpatient hospital stay is exempt from this prior authorization requirement.

II. Clinical Medical Necessity Criteria

To receive approval for a lumbar spine MRI, the ordering provider's submitted documentation must satisfy at least one of the following clinical thresholds:

Conservative Management Failure: The patient has experienced severe, non-radiating low back pain for greater than 6 weeks AND has documented failure of at least two conservative treatment modalities. Eligible modalities include a minimum 21-day trial of prescription NSAIDs, physical therapy (minimum 4 supervised sessions), or directed chiropractic care.

Radiculopathy with Functional Impairment: The patient exhibits objective signs of nerve root compression (e.g., altered deep tendon reflexes, dermatomal numbness, or asymmetric muscle weakness) that correlates with functional limitations in daily activities.

Red Flag Progressive Symptoms: The patient presents with rapidly progressive neurological deficits, suspected spinal cord compression, or a recent history of high-velocity trauma.

III. Administrative & Appeal Rules

Retroactive authorization requests will not be reviewed. Failure to secure an approved prior authorization token before the date of service triggers an automatic administrative denial. Providers forfeit all standard peer-to-peer and formal appeal rights for claims denied under this administrative rule; the financial liability rests solely with the rendering facility.